

إسم المريض رابعياً : سعه محمد عبدالقادر احمد

الحي / المدينة : سمسطا

المحافضة : بنى سويف

رقم البطاقة : 26508042200125 رقم التلفون : 01124196101

العمر(تاريخ الميلاد) : 04-08-1965 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR1-E
جورستان
روستای پلانی
لندی حسن
موسین رمضان
صحة البیابان المرفقة
للجنة و مدین

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: _____ التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة الاياماب المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :-

المستشفى : ----- قسم :

جهة العلاج : ----- نفقات :

ملاحظات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: سعدة محمد عبدالقادر احمد National ID: 26508042200125	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Saada Mohamed Abd El-Kader
Date: 2026-09-13
Referred by: Prof. Dr. no doctor
Hospital: Gustave Roussy International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 60-year-old female with negative family history complaining of left breast mass.

Findings:

Right breast:

- Scattered fibroglandular breast parenchyma (ACR B)
- No speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with no suspicious solid or cystic lesions
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs

Left breast:

- Scattered fibroglandular breast parenchyma (ACR B)
- Scattered fibroglandular breast parenchyma with spiculated outlines and Retroareolar irregular speculated high density mass lesion with surrounding distortion yet no suspicious microcalcifications.
- **Ultrasound** demonstrated Retroareolar breast 12 o'clock irregular spiculated heterogeneous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and minimal internal vascularity on color Doppler study measuring about 2.4 x 2.8 cm.
- Retracted nipple.
- Non-specific axillary LNs.

Conclusion:

- **Right breast (BIRADS 1):** Unremarkable study.
- **Left breast (BIRADS 4C):** suspicious looking Lesion as described.

Recommendation:-

- For biopsy and histopathological correlation.

Best Regards,
Dr.Nehal Fouda,MD

Pat. Name: <i>سیدہ محمد عبداللہ احمد</i>	Pat. ID: 615278
Ref. Phy.:	Study Date 12/09/2026

Digitalized Low Dose Soft Tissue Mammography With Complementary Breast US Examination Of Left Breast Revealed:

- ACR B: Mixed fibro-fatty-glandular breast parenchyma.
- An irregular shaped speculated hyperechoic lesion is seen at the upper aspect of the left breast corresponding by US to an irregular shaped globular shaped heterogeneous hypoechoic lesion is seen at 12 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 25x28 mm.
- No pathological microcalcification clusters.
- cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- No pathological axillary, supra or infra clavicular lymph nodes.

Opinion:

- Left breast malignant looking lesion, as described (BIRADS IV C); for pathological correlation.

Best regard

Dr. Huda Mostafa, FEB

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

<i>BIRADS 0:</i>	<i>Needs further investigations.</i>
<i>BIRADS 1:</i>	<i>Normal findings.</i>
<i>BIRADS 2:</i>	<i>Benign.</i>
<i>BIRADS 3:</i>	<i>Probably benign.</i>
<i>BIRADS 4:</i>	<i>Probably malignant.</i>
<i>BIRADS 5:</i>	<i>Malignant.</i>
<i>BIRADS 6:</i>	<i>Proven pathologically malignant</i>

Hot Line

010 2300 1755

خدمة العملاء والطوارئ 24 ساعة

٢٠٢٠/١٠ ١٦٥٠٨٠٤٢٢٠٠١٢٥
رَبِّهِ مَنْزِل
أَنْثَى مُسَلِّمَةٌ أَرْمَلَةٌ
البطاقة سارية حتى ٢٠٢٧/١٠/٠٥ ٦



late m ID - Back
DOCTYPED ID - Back

Patient Name	سعد محمد عبد القادر	Case No.	384568
Sex/Age	Female 61 Y	Patient No.	28937
Registered Date	14/9/2026 12:39:15 PM	Referred By Dr.	Interventional Radiology
Validation Date	20/9/2026 10:47:09 AM	Branch	Gustave Roussy
Reporting Date	21/9/2026 12:18:34 PM	Pathology No.	2926/26

Pathology Report

Clinical Data: Left breast 12 O'clock mass (BIRADs 5).

Nature of Specimen: Tru-cut biopsy, collected on 14/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:

-Three fragmented grey, white tissue cores, the longest is 1 cm, totally submitted.

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma with mucinous differentiation.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Focal low-grade DCIS (Solid pattern).
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with mucinous differentiation, Grade 2.
- Focal low grade ductal carcinoma in situ (Solid pattern).
- ** Case for further (ER, PR, Her2 & Ki67).

Signature

Prof. Dr. Maha Guimei, M.D, PhD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

شع المختبر
د/إسماعيل علي عبد الحليم
د/إسماعيل علي

مختبر الدم
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مختبر الدم
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2026-09-12
2026-09-12
د/إسماعيل علي

مختبر الدم
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مختبر الدم
202604001797
26036145
56 Year / Female

مختبر الدم
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مختبر الدم

Hematology Unit

Reference Range

COMPLETE BLOOD PICTURE

Haemoglobin	12.9	g/dL	11.5	15.5
Red Cell Count	4.5	millicmm	4.0	5.2
Haematocrit (PCV)	42.6	%	36.0	45.0
Red cell Indices				
MCV	94.0	fL	82.0	99.0
MCH	28.5	pg	27.0	33.0
MCHC	30.3	g/dL	31.0	37.0
Total & Differential Leucocyte Count				
Total Leucocyte Count	7.0	$\times 10^3/\text{cmm}$	3.8	11.0

Differential Leucocyte Count

	Relative	Absolute		
Neutrophils	1	%	0.07 $\times 10^3/\text{uL}$	
Band	72	%	5.04 $\times 10^3/\text{uL}$	2000.0
Segmented	17	%	1.19 $\times 10^3/\text{uL}$	1000.0
Lymphocytes	7	%	0.49 $\times 10^3/\text{uL}$	70.0
Monocytes	3	%	0.21 $\times 10^3/\text{uL}$	0
Eosinophils	0	%	0.00 $\times 10^3/\text{uL}$	0
Basophils				100.0
Others				
Platelet Count	L	136.0	$\times 10^3/\text{cmm}$	150.0
				450.0

Comments:

-Relative lymphopenia.
-Mild thrombocytopenia



Ahmed Gomaa

Dr. Ahmed Gomaa
MSc of clinical pathology

مختبر الدم بالتحليل

Patient's Name:

Saada Mohamed Abd El Kader

Date:

2026-09-13

Referred by:

Prof Dr. no doctor

Hospital:

Coastave Roussey International Egypt

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- **Left breast (BIRADS 4C):** suspicious looking Lesion as described.

Recommendation:-

- **For biopsy and histopathological correlation:-**

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Dr.Nehal Fouda,MD

Pat. Name: محمد عبد الله احمد	Pat. ID: 615278
Ref. Phy.	Study Date 12/09/2026

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Opinion:

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Hot Line

010 2300 1755

خدمة العملاء والطوارئ 24 ساعة

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سعد
محمد عبدالقادر احمد
شارع الجلاء - دشتوط
مركز سمسطا - بنى سويف

٢٦٥٠٨٠٤٢٢٠٠١٢٥

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د/إسماعيل علي

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د/إسماعيل علي

2026-09-12
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د/إسماعيل علي

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مختبر الدم
202604001797
26036145
56 Year / Female

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Hematology Unit

Reference Range

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Comments:

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Ahmed Gomaa

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MSc of clinical pathology

مختبر الدم بالتحليل

Patient's Name:

Saad Mohamed Abd El Kader

Date:

2026-09-13

Referred by:

Prof Dr. no doctor

Hospital:

Coastal Roussey International Egypt

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جمهورية مصر العربية
بطاقة تحقيق الشخصية

سعد
محمد عبدالقادر احمد
شارع الجلاء - دشتوط
مركز سمسطا - بنى سويف

٢٦٥٠٨٠٤٢٢٠٠١٢٥

١٠٦٧٩٢٠٤٤



Patient Name	سعد محمد عبد القادر	Case No.	384568
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Ahmed Gomaa

Dr. Ahmed Gomaa
MSc of clinical pathology

مختبر الدم بالتحليل

Patient's Name:

Saadat Mohamed Abd El Kader

Date:

2026-09-13

Referred by:

Prof Dr. no doctor

Hospital:

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Pat. Name: محمد عبد الله احمد	Pat. ID: 615278
Ref. Phy.	Study Date 12/09/2026

Digitalized Low Dose Soft Tissue Mammography With Complementary Breast US Examination Of Left Breast Revealed:

- ACR B: Mixed fibro-fatty-glandular breast parenchyma.
- An irregular shaped speculated hyperechoic lesion is seen at the upper aspect of the left breast corresponding by US to an irregular shaped globular shaped heterogeneous hypoechoic lesion is seen at 12 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 25x28 mm.
- No pathological microcalcification clusters.
- cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- No pathological axillary , supra or infra clavicular lymph nodes.

Opinion:

- Left breast malignant looking lesion , as described (BIRADS IV C); for pathological correlation.

Best regard

Dr. Huda Mostafa , FEB

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0:	Needs further investigations.
BIRADS 1:	Normal findings.
BIRADS 2:	Benign.
BIRADS 3:	Probably benign.
BIRADS 4:	Probably malignant.
BIRADS 5:	Malignant.
BIRADS 6:	Proven pathologically malignant

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خدمة العملاء والطوارئ 24 ساعة

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بطاقة تحقيق الشخصية

سعد
محمد عبدالقادر احمد
شارع الجلاء - د. شطوط
مركز سمسطا - بنى سويف

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Patient Name	سعد محمد عبد القادر	Case No.	384568
Sex/Age	Female 61 Y	Patient No.	28937
Registered Date	14/9/2026 12:39:15 PM	Referred By Dr.	Interventional Radiology
Validation Date	20/9/2026 10:47:09 AM	Branch	Gustave Roussy
Reporting Date	21/9/2026 12:18:34 PM	Pathology No.	2926/26

Pathology Report

Clinical Data: Left breast 12 O'clock mass (BIRADs 5).

Nature of Specimen: Tru-cut biopsy, collected on 14/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:

-Three fragmented grey, white tissue cores, the longest is 1 cm, totally submitted.

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma with mucinous differentiation.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Focal low-grade DCIS (Solid pattern).
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with mucinous differentiation, Grade 2.
- Focal low grade ductal carcinoma in situ (Solid pattern).
- ** Case for further (ER, PR, Her2 & Ki67).

Signature

Prof. Dr. Maha Guimei, M.D, PhD.



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